

VitalisCare *Sync*

A strong hospice scheduling engine with live HCHB integration and a thoughtful stance on keeping schedulers in charge. Home health is almost entirely ahead of it: authorization, the episode and LUPA, therapy disciplines and patient communication are all still to be built, and the product was demoed on hospice data.

SCORECARD GOING IN 67 Consider · no open stop-checks	LIVE FOOTPRINT 3 orgs Sync fully live, ~6-7k patients. Largest client 3,500 ADC	GUIDE QUESTIONS 5 / 18 answered in full; 7 partly; 6 unanswered or not asked	NEXT TOUCHPOINT Oct 5 Week of the second virtual call. On-site summit week of Oct 19
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Six things that matter

- 01 **It is a hospice product, and they said so.** “Our Sync tool has been developed primarily for hospice.” The demo used hospice disciplines (aide, social work, chaplain), hospice compliance alerts (face-to-face, RN supervisory gaps) and hospice CMS benchmarks. 0:23:57
- 02 **Their own gap list is authorization and patient communication.** Compassus added LUPA and episode management, therapy productivity, territories and two-way patient scheduling. Vitalis calls each one “decision-tree logic” or “weeks, not months.” None of it exists yet. 1:21:32
- 03 **HCHB freshness is the real constraint, and it is on our side.** Their typical client ships HCHB logs every ~10 minutes. Compassus ships every ~3 hours, which they called too stale for call-off recovery. Writes back through RPA take seconds to minutes. 0:46:46
- 04 **Advise-not-decide is a principle, not a phase.** Every recommendation is accept-or-dismiss. Overrides require a reason, which feeds the model. This fits our Assist boundary and our clinician culture. 0:59:04
- 05 **Exception handling is still a design, not a product.** The Call-Off Center, which makes the 3-hour lag survivable, is “in design, slated for release before the end of the year.” It was shown only as slides. 1:04:48
- 06 **The clinician app is a GPS tracking app.** Sync360 monitors the workday, reroutes in real time and powers a performance review of reported versus tracked mileage. It is powerful for oversight and a real adoption question for tenured clinicians. 1:13:48

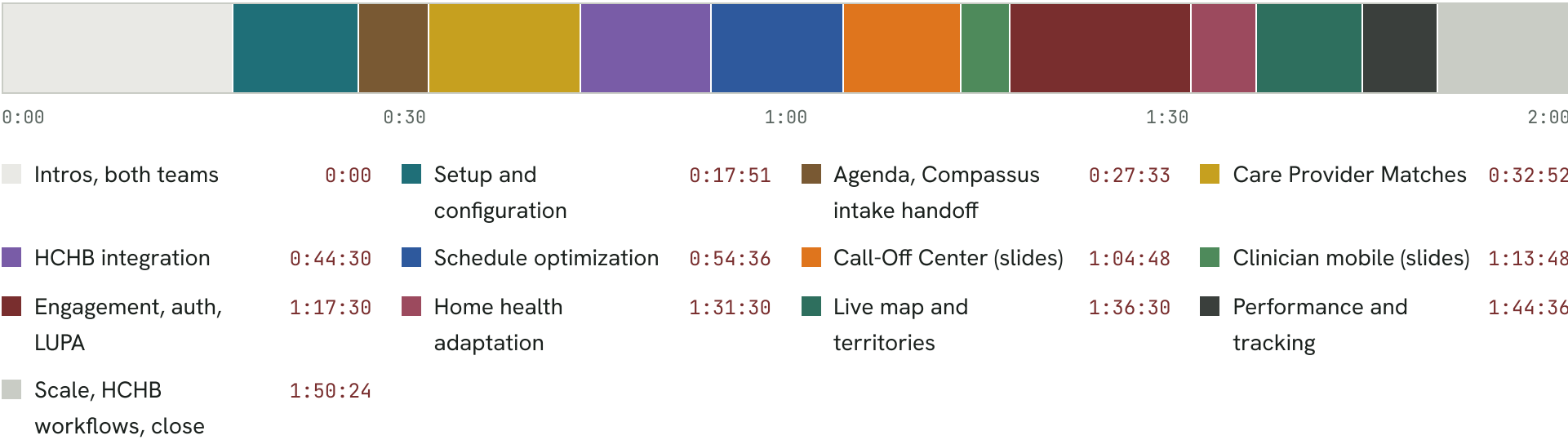
SUGGESTED SCORECARD MOVEMENT

Hold at Consider. Do not advance on product, advance on partnership.

The demo confirmed the scorecard’s central doubt instead of resolving it. What it added is a credible design-partner posture: a fast team, open to our workflow, and willing to put engineering against it. Advance to the on-site only if the second call produces dates and a scope for the home health build, plus a named plan for HCHB data cadence.

Architecture & Continuity (security, uptime) was on their agenda and never presented. Treat it as open.

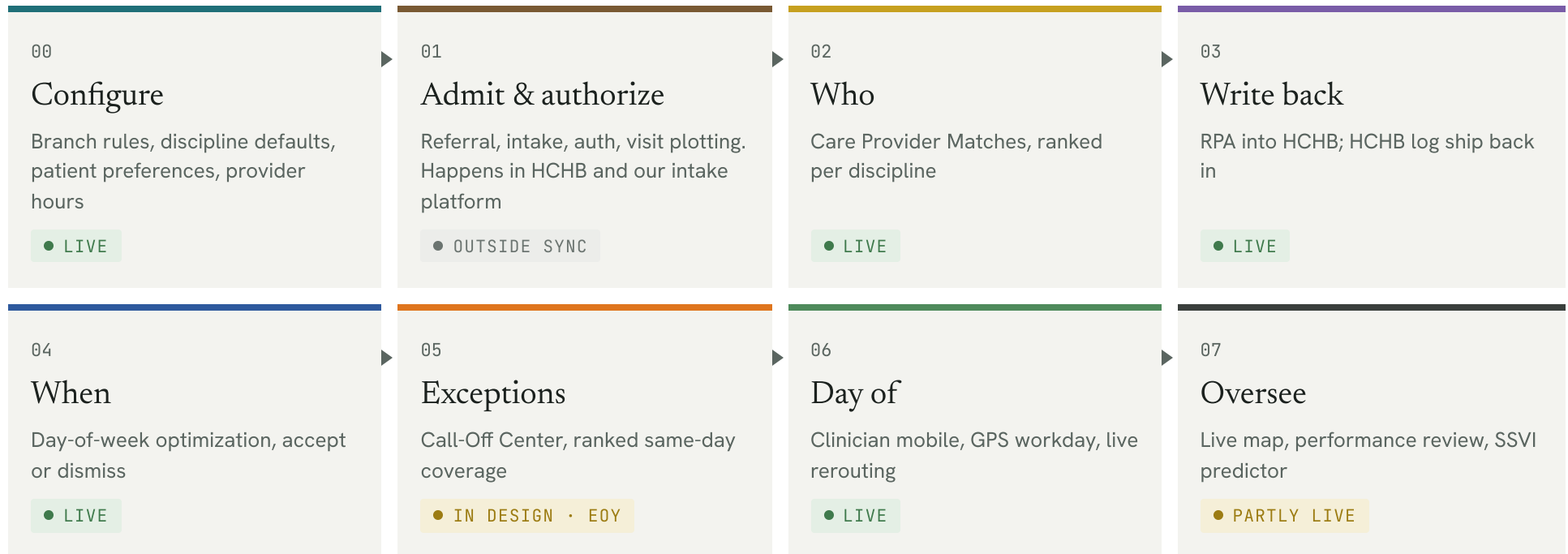
The shape of the call



About 45 minutes was live software. The rest was slides and discussion, which is where most of the home health substance came out.

From admitted patient to audited day

Vitalis frames scheduling in three questions: *who* visits, *when*, and *by what route*. Sync picks up only after admission and authorization are complete, so everything upstream of stage 2 stays with Compassus and HCHB.



● LIVE

demonstrated in software

● IN DESIGN

shown as slides or partly built

● NOT BUILT

named as a gap

● OUTSIDE SYNC

not their workflow

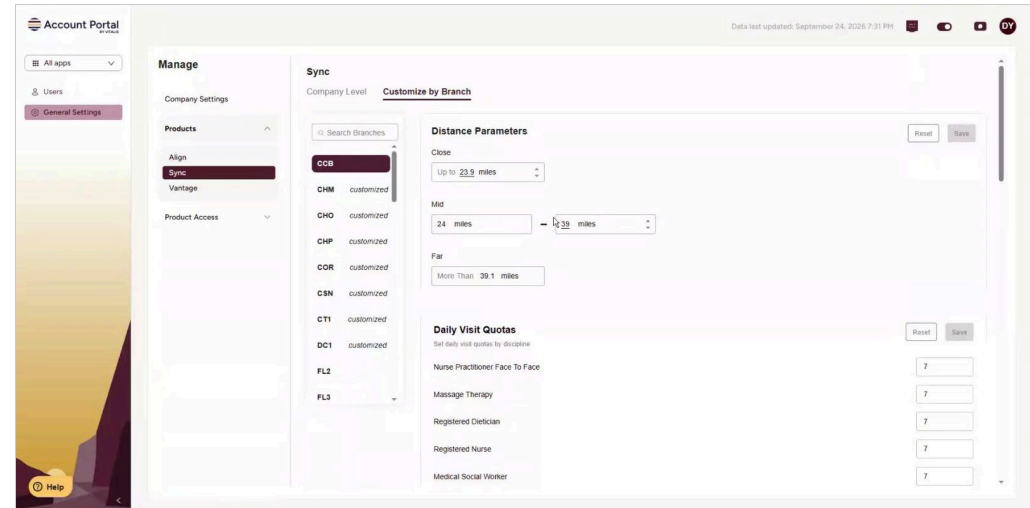
Rules set once per branch and discipline, then per person

WHAT IT DOES

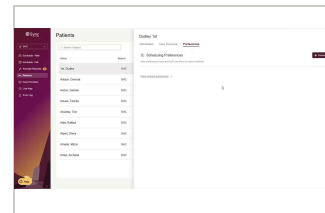
- **Patient preferences** as free text (language, days, hours, discipline). They can be entered by schedulers on the web or by clinicians in the app, and feed the algorithms.
- **Care provider profile:** weekly hours, weekends, long-term-assignment eligibility (to keep per diem or on-call staff out), date overrides and PTO.
- **Branch settings in the Account Portal:** distance bands (demo branch: close ≤ 23.9 mi, far > 39.1 mi), daily visit quotas per discipline, RN-or-LPN substitution for SN orders, and matching scope (the patient's branch, associated branches, any branch or a custom set).
- **New-worker ramp:** how long someone counts as new (default 2 weeks) and how fast their load ramps. Full-time and part-time hour definitions. Standard weekly schedule per discipline.

FOR COMPASSUS

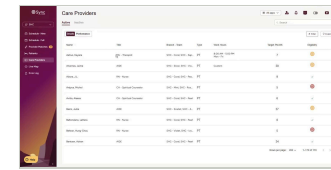
- Colin asked whether new clinicians can be kept off specific visit types such as SOC. **Not today;** ramp is by productivity only. They offered to add it. 0:25:36
- With 80+ branches, per-branch configuration is real implementation work. Ask who configures it and how long it takes.
- The settings list is hospice-shaped: it includes music therapy, massage and Reiki, and no PT, OT or SLP defaults were shown.



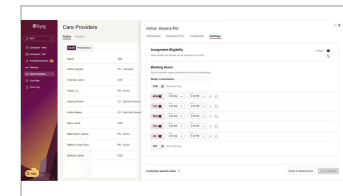
0:23:22 Branch settings: distance bands and daily visit quotas by discipline



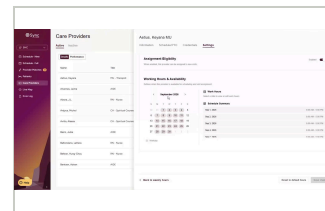
0:19:10 Patient scheduling preferences



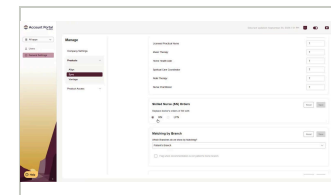
0:20:04 Care provider roster: title, branch and team, work hours, target visits, eligibility



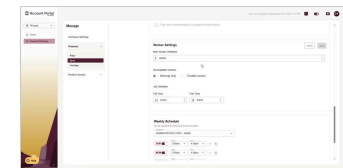
0:20:55 Provider assignment eligibility and weekly hours



0:21:31 Date-specific hours and PTO



0:24:01 SN orders to RN or LPN; matching by branch



0:25:25 New-worker definition, incompatible workers, full-time and part-time hours

Sync starts where our referral work ends

WHAT WAS SAID

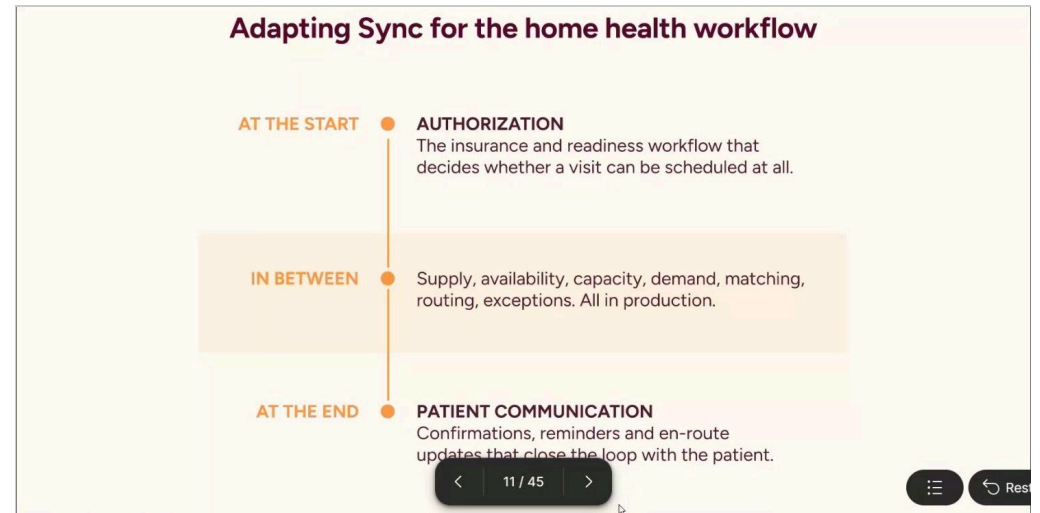
- Vitalis: “We start the scheduling process once the patient is fully admitted” and authorization is complete. Authorization is not a state Sync tracks.
- Michael described the new Compassus referral and intake platform: marketers, intake, insurance and pre-SOC operators produce a complete, valid referral before it reaches HCHB. It carries addresses, disciplines, auths and visit plans as discrete data.
- Michael’s idea is to divert that data to the scheduling tool while it is in transit to HCHB, and to use capacity insight in the accept or decline decision. Vitalis: “very interesting... definitely something we can entertain.”
- Visits are plotted on the HCHB calendar by intake teams today. Sync optimizes after that. Plotting through write-back has been “explored but not implemented.” 1:52:39

FOR COMPASSUS

- Common question 1, the referral decision, has no product answer. The front door of our operating model sits outside Sync.
- The intake-to-Sync feed is our idea, not their roadmap. If we want it, we will be specifying and co-funding it.

“We’re looking at this like two sides of a coin. First we solved referral management... now how can we use that information to feed capacity management and scheduling.”

Michael Thompson, Compassus • 0:31:13



1:21:34 Vitalis’s own framing: authorization at the start and patient communication at the end are the home health build. Everything in between is “in production.”

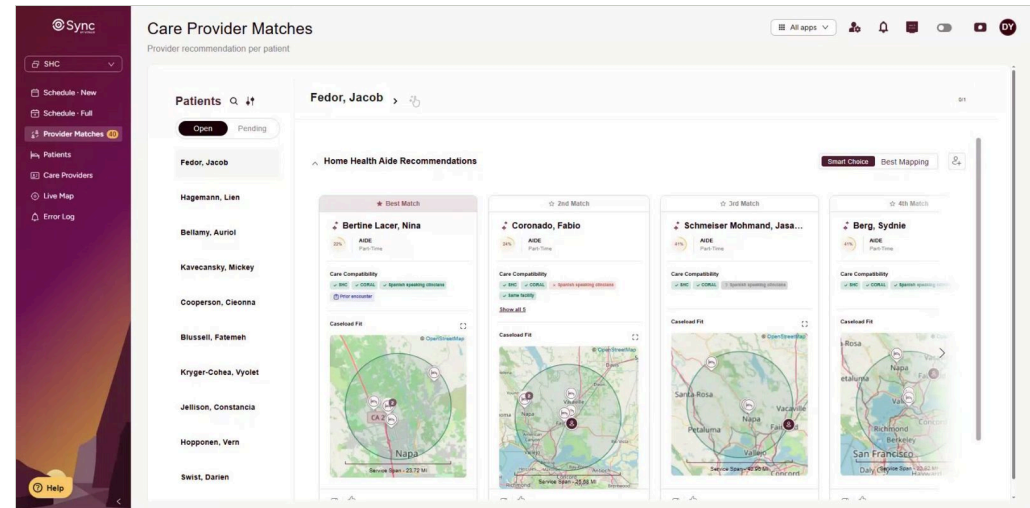
Care Provider Matches: ranked long-term assignment per discipline

WHAT IT DOES

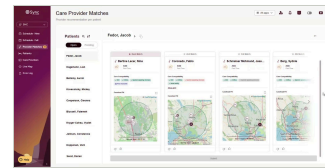
- A newly admitted patient appears in a queue. Sync shows the top four providers for each discipline (the hospice default is aide, RN, social work and chaplain). It can be limited to disciplines with open orders.
- There are two lenses. **Best Mapping** ranks on location and caseload geography first. **Smart Choice** balances location against capacity and productivity, and most customers use it.
- Each card shows a productivity %, a **caseload-fit map** with service span in miles, compatibility chips (branch, team, language, prior encounter, same facility) and a short AI rationale.
- An override requires a reason. That reason is both a feedback loop into the model and an audit trail for leadership.
- Submit moves the patient to Pending while RPA writes the assignment to HCHB. Sync polls until the write is confirmed.

FOR COMPASSUS

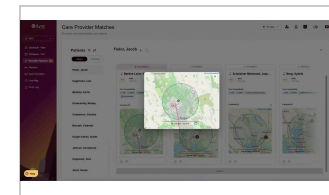
- Territories:** Colin asked about ZIP-based territories. Sync uses teams and branches, not territories. If we don't encode territories in HCHB, they would need to understand how we categorize them. 0:36:43
- Clinician-facing reasoning:** Colin asked whether clinicians see the "why" behind an assignment outside their usual area. **Not built;** Vitalis liked the idea. 0:41:27
- Productivity points can be shown against target, which Paige called "a really great touch."



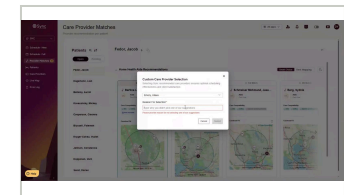
0:32:52 Care Provider Matches: Smart Choice, four ranked aides, each with a caseload map and compatibility chips



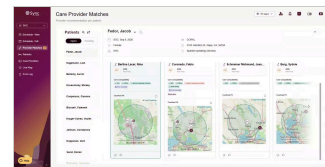
0:37:16 Compatibility chips: branch, team, Spanish-speaking, prior encounter, same facility



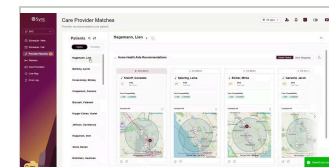
0:38:22 Caseload map expanded, service span 23.72 mi



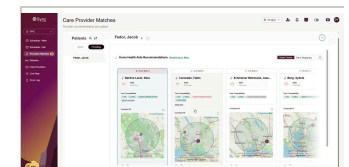
0:39:55 Override: custom provider selection requires a reason



0:41:34 Patient context: SOC date, branch, address, language preference



0:43:25 Next patient. The submitted match writes back to HCHB



0:45:58 Pending: still editable until the RPA write-back is confirmed

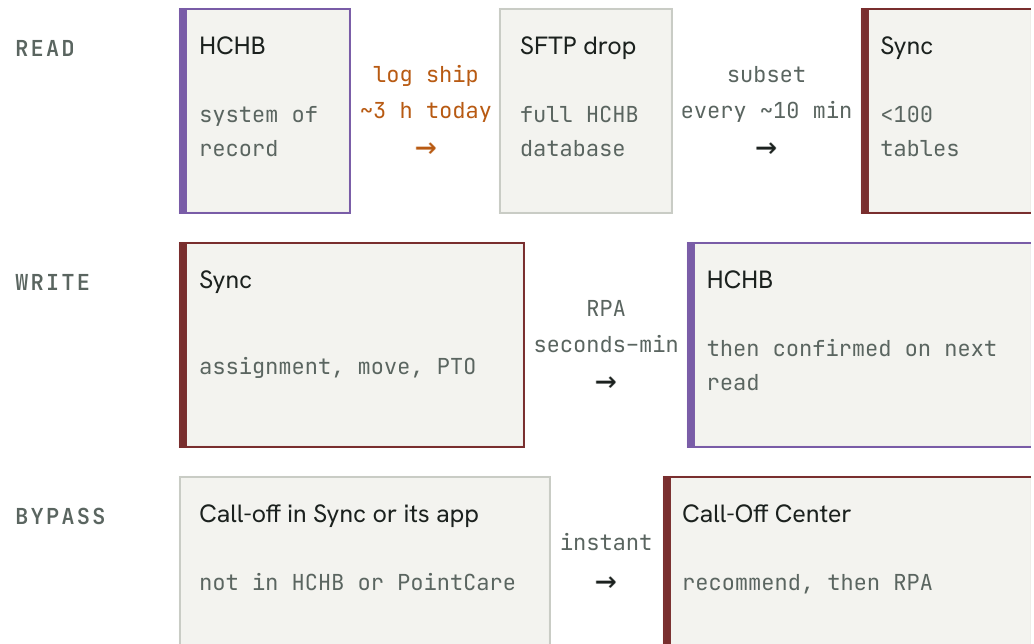
HCHB in by log shipping, out by RPA

HOW IT WORKS

- **Read:** the HCHB log ship arrives on SFTP, loads into a full database, and a subset of fewer than 100 of HCHB's thousands of tables is refreshed every ~10 minutes. Very large clients run Vitalis's subset script on their own side. One client streams from an HCHB database mirror.
- **Write:** RPA (software robots working HCHB's screens), which Vitalis calls "very robust" and uses across products. Seconds to minutes per write.
- Vitalis is in partnership talks with HCHB for 2027. Today they rely on log shipping.

FOR COMPASSUS

- **Minimum cadence was not committed.** "It depends on what you expect." One client is fine hourly; others say 10 minutes is too slow. Reassignment is where it bites.
- **Double entry is the failure mode.** Schedulers who don't trust the lag redo the change in HCHB and create conflicts. That makes adoption discipline part of the integration design.
- Compassus has cut HCHB refresh from 6 hours to 3 for another AI project and will explore going faster. A broader data modernization "probably solves all of our problems" but takes a while.
- Not asked: what happens when HCHB changes a screen and the RPA breaks. Ask on the second call. The same fragility was flagged for Arya's screen automation.



3 h

Compassus's current HCHB log-ship interval. A call-off entered in PointCare or HCHB reaches Sync up to three hours later, "too late to reschedule the visit." The workaround is to trigger call-offs inside Sync, or have Sync listen to PointCare actions (they do this in other apps). Either way, schedulers and clinicians have to change where they act. 1:09:03

Schedule optimization, one suggestion at a time

WHAT IT DOES

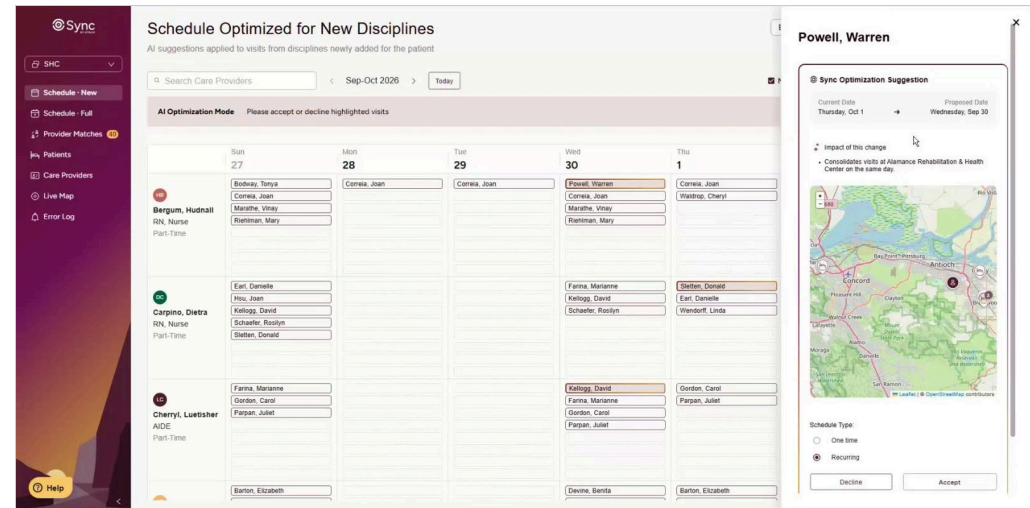
- A week grid per clinician flags visits that aren't on their best day. For example, it moves Powell, Warren from Thursday to Wednesday because the provider is already at that facility that day. Each suggestion shows the reasons and that day's caseload map.
- Accept once or recurring, or decline. Manual drag-and-drop warns on out-of-window visits, expired licenses and same-day duplicates.
- The provider panel shows the 30-day caseload map, visit frequency and certification alerts (yellow when expiring, red when expired).
- The original HCHB schedule can be viewed side by side. There is also PTO entry, export for facility communication, and a Notification Center of compliance alerts (hospice: face-to-face and RN supervisory gaps).
- **Schedule Full** reshuffles a whole month for support staff. Vitalis prefers **small incremental shifts** that move caseload centroids over time.

FOR COMPASSUS

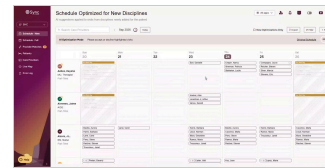
- Hospice frequencies are weekly or monthly. There was **no SOC window, recert window or LUPA threshold** in the logic shown. Compassus raised LUPA directly; Vitalis said it is "flagged... a gap." Front-loading logic already exists and could be reused. **1:24:45**
- Home health compliance alerts (OASIS timepoints, recert, missed-visit risk) would all be new.

"Retaining that power balance with your schedulers making final decisions empowers them rather than paralyzes them."

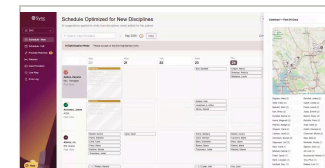
Dina Yankelewitz, CEO • 0:59:04



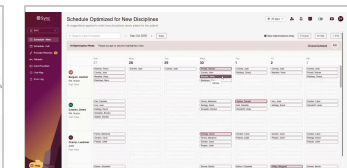
0:59:13 Optimization suggestion: Thursday to Wednesday to consolidate visits at a facility. One-time or recurring; accept or decline



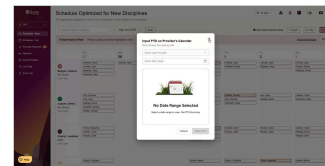
0:55:16 AI Optimization Mode, week by clinician



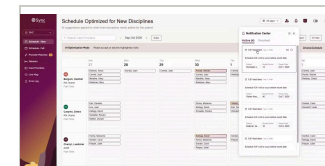
0:56:01 Provider caseload, past 30 days



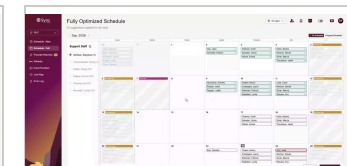
1:01:43 Manual moves with conflict flags



1:02:46 PTO entry greys out that day's visits and routes them to the Call-Off Center



1:03:52 Notification Center with hospice face-to-face alerts



1:43:28 Schedule Full: monthly reshuffle for support staff

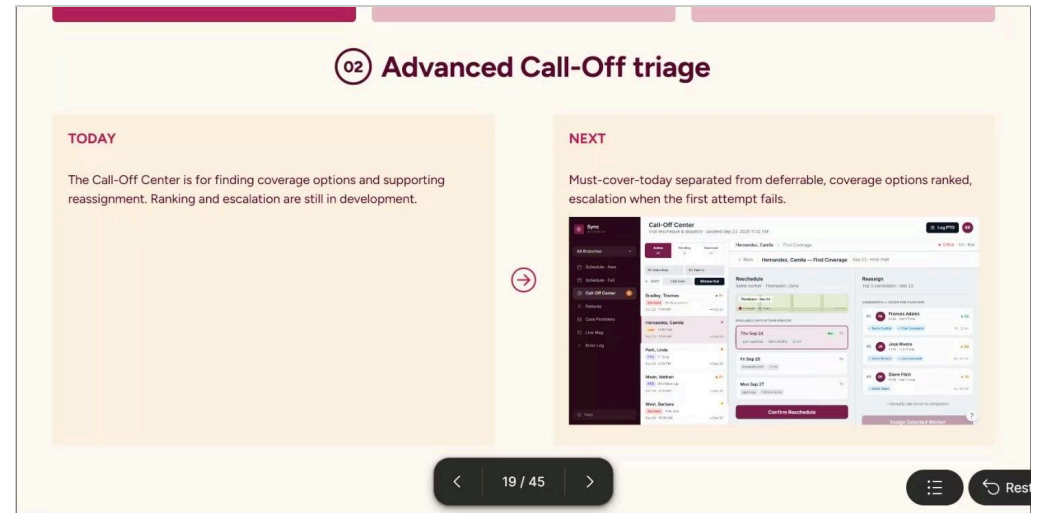
Call-Off Center: the piece that makes the lag survivable

AS DESCRIBED

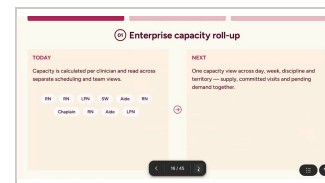
- A queue ranked by urgency. It pulls visits from PTO, HCHB and PointCare declines or late statuses, and weighs visit windows and compliance (hospice HOPE visits, for example).
- It recommends either rescheduling with the same provider or replacing them. Trade-offs are shown: productivity, drive time, overtime risk. It keeps the same replacement across a multi-day absence for continuity.
- It writes back to HCHB and pushes the change to the clinician's phone at once.

FOR COMPASSUS

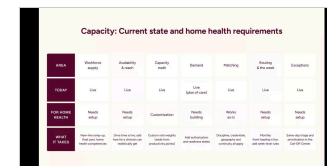
- **Their slides contradict each other.** The capacity slide marks Exceptions as "Live"; the roadmap slide says ranking and escalation "are still in development." Believe the roadmap slide.
- Common question 6, a Tuesday that goes wrong, was described but not run live. "What happens when nobody takes the visit" was not answered.
- Compassus said it does not want a third-party call center (Vitalis's term for the Call-Off Center) "right out of the gates" but wants all the functionality. Clarify what that means before the next call.



1:11:55 Roadmap slide: Call-Off triage. Today, coverage options; next, must-cover-today separated from deferrable, with escalation



1:05:07 Roadmap: enterprise capacity roll-up. Today capacity is per clinician



1:34:28 Capacity for home health: Demand "needs building" (authorization and readiness states)

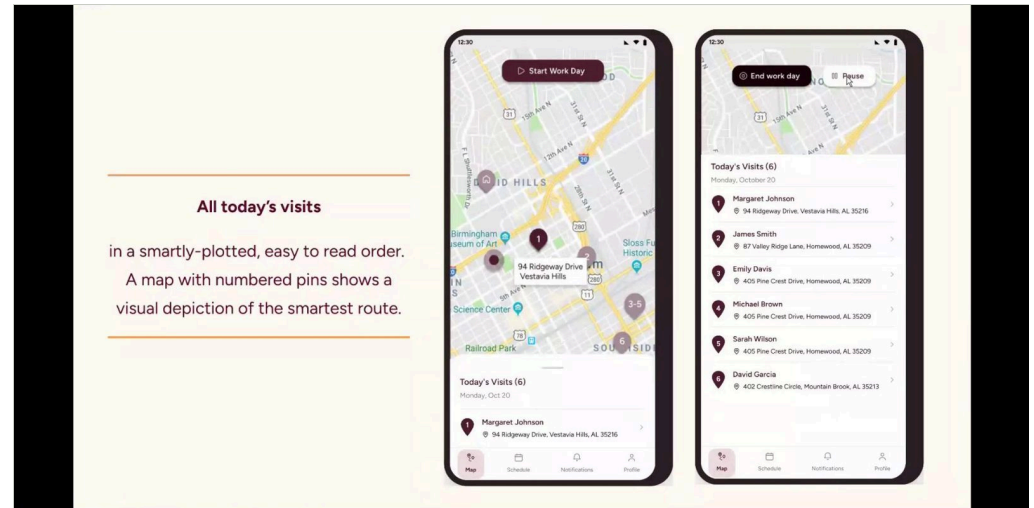
Clinician mobile (Sync360)

WHAT IT DOES

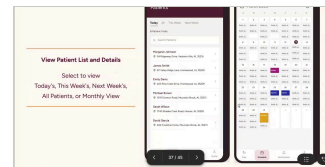
- The clinician starts, pauses and ends the workday; GPS monitors visit completion only while the workday is on.
- Today's visits come as a routed list with a map, and hand off to Google Maps or Waze. The route **recalculates in real time** as visits are added or removed.
- Today, week or month views; patient details; adding patient preferences; notifications of schedule changes; setting their own working hours.
- Time off comes from HR systems (iSolved integration live; Workday to be explored) so nothing is entered twice.

FOR COMPASSUS

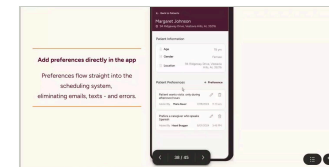
- Clinicians would still document in PointCare, so this is a **second app** (VitalisCare question 8). Declining a visit from the app depends on the Call-Off Center.
- Compassus's north star is no work inside HCHB and one screen for schedulers and clinicians. Vitalis says no other organization has asked for that. "We've been begging for that."
- Colin pressed on day-before confirmations: 4-5pm calls and texts, replies at 9pm. The app has nothing for this yet. See the engagement row below.



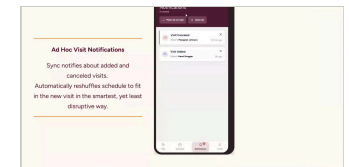
1:14:43 Today's visits in route order with numbered map pins



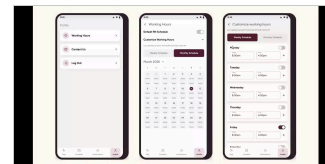
1:15:07 Patient list and calendar views



1:15:16 Patient preferences captured in the field



1:15:25 Added or canceled visit notifications



1:15:46 Clinician sets their own working hours

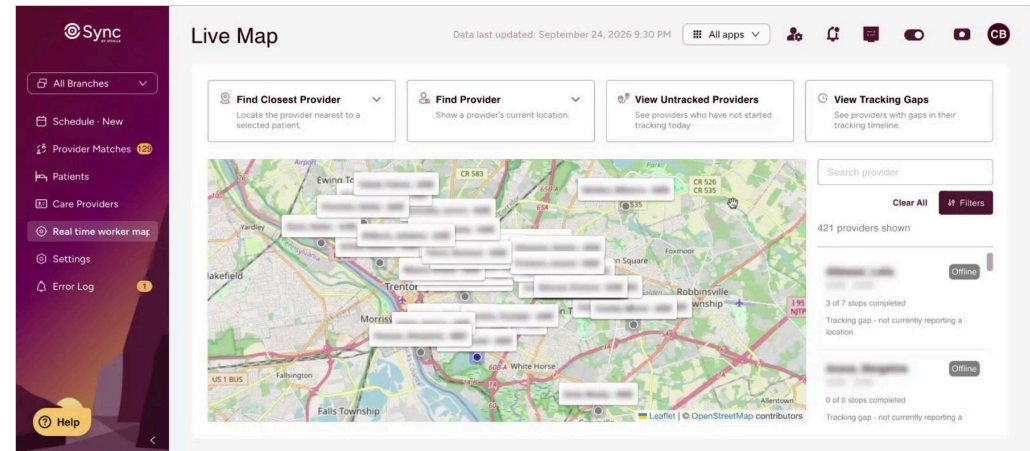
Live map, performance review and predictive benchmarks

WHAT IT DOES

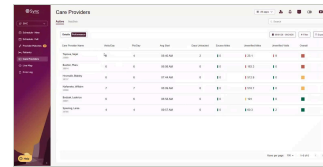
- **Live Map** (real data, names blurred): where every clinician is now, their progress through the day, the closest provider to a patient, untracked providers and tracking gaps.
- **Care Provider Performance**: visits per day, average start time, flags when a device is switched off, and a colour-coded triage. Each day can be reviewed for reported versus tracked mileage, time and duration mismatches, and route adherence on a map.
- **SSVI Predictor** (designed, not live) tracks CMS hospice service benchmarks.

FOR COMPASSUS

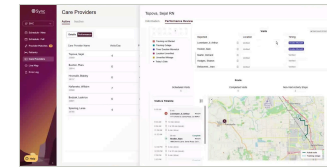
- **Territories are the gap.** Colin wants a branch-director view of patient and provider caseload by territory that shifts with referral patterns, because territory is “the start of capacity.” The live map is day-of only. Vitalis: “it wouldn’t be hard to build.” 1:40:42
- The tracking and discrepancy layer is framed as accountability. For tenured, salaried home health clinicians it needs deliberate change management and probably HR and legal review.
- SSVI and the benchmarks are hospice measures. There is no HHVBP or OASIS outcomes equivalent.



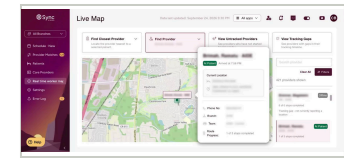
1:38:13 Live Map, real data with names blurred: find the closest provider, untracked providers, tracking gaps



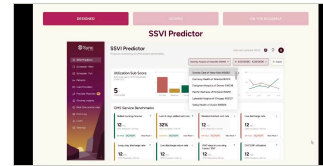
1:47:10 Performance: visits per day, average start, device-off flags



1:48:43 Day review: reported versus verified visits and route adherence



1:39:13 Find a provider's current location and route progress



1:51:55 SSVI Predictor, designed: hospice CMS benchmarks

What it would take to make this ours

AREA	STATUS FOR HOME HEALTH	WHAT IT TAKES (VITALIS)	WHAT COMPASSUS ADDED IN THE ROOM
Authorization & readiness	● NEEDS BUILDING	Add authorization and readiness states. Today they only flag moves outside an auth window using HCHB data.	Framed as a compliance constraint, not auth automation: which patients can be scheduled before auth and which must wait. "A basic decision tree." 1:23:33
Episode, LUPA & recert	● NEEDS BUILDING	Not on their slide. Front-loading logic exists.	Episodic payment and LUPA flags for over- and under-utilization against the plan of care. "That can blow up our P&L really quickly." 1:24:45
Therapy disciplines	● CUSTOMIZATION	Productivity from HCHB points or visit count per discipline, prorated. "We don't really deal with therapies in hospice."	No PT, OT or SLP evaluation-driven frequency or sequencing discussed.
Workforce supply	● NEEDS SETUP	New-hire ramp-up, float pool, home health competencies.	Keep new hires off SOC visits for a period.
Availability, reach, routing	● NEEDS SETUP	Drive time is live; add realistic reach and week-level rules. Home, branch or horseshoe start points exist.	Territory-based assignment and a branch territory map.
Matching	● WORKS AS-IS	Discipline, credentials, geography and continuity all apply.	Clinician-facing explanation of why they got a patient.
Exceptions	● NEEDS SETUP	Same-day triage and prioritization in the Call-Off Center.	Home health visit windows and discipline priority for coverage.
Patient engagement	● NEEDS BUILDING	Confirmations, reminders and en-route updates. Deprioritized in hospice over HIPAA and the caregiver relationship.	Two-way scheduling with patients. Clinicians spend 30-60 min a day on confirmations; 10,000 clinicians across Compassus. Even text-only that removes half of it is a large win. 1:26:12

Engagement: Current state and home health requirements

In hospice, the contact is the caregiver, not the patient — and a significant share of patients are in facilities. Visit patterns are steady and reschedules are rare. Home health reverses all of that, which is why this is a build rather than a setting.

AREA	Before the visit	When plans change	Incentives & offers	Across the care team
TODAY	Clinician side live	Live	Partly live	Live
FOR HOME HEALTH	Needs building	Needs building	Needs building	Works as-is
WHAT IT TAKES	Patient confirmations, reminders, en-route updates	Same-day reassignment is live; add patient-facing notice	Open and hard-to-fill visits surface today; no differential offers	Multi-discipline coordination, HCHB sync, field and office visibility

1:28:13 Engagement: every home health column “needs building” except coordination across the care team

Capacity: Current state and home health requirements

AREA	Workforce supply	Availability & reach	Capacity math	Demand	Matching	Routing & the week	Exceptions
TODAY	Live	Live	Live	Live (plan of care)	Live	Live	Live
FOR HOME HEALTH	Needs setup	Needs setup	Customization	Needs building	Works as-is	Needs setup	Needs setup
WHAT IT TAKES	New-hire ramp-up, float pool, home health competencies	Drive time is live; add how far a clinician can realistically get	Custom visit weights (aside from productivity points)	Add authorization and readiness states	Discipline, credentials, geography and continuity all apply	Monthly front-loading is live; add week-level rules	Same-day triage and prioritization in the Call-Off Center

1:34:28 Capacity: “Live” today everywhere; needs setup or building for home health

Question by question

• ANSWERED 5 • PARTLY 7 • NOT ANSWERED 3 • NOT ASKED 3

QUESTION	STATUS	WHAT WE HEARD	WHEN
Common 01 The referral decision	• NOT ANSWERED	Sync starts after admission and auth. There is no referral-against-capacity check. Branch capacity roll-up is roadmap. The idea of feeding the Compassus intake platform into Sync came from Michael, not Vitalis.	0:28:42
Common 02 The episode, not the visit	• NOT ANSWERED	No SOC or recert window or LUPA logic shown. Compassus raised LUPA; Vitalis acknowledged the gap and pointed to existing front-loading logic.	1:24:45
Common 03 HCHB freshness, with a number	• ANSWERED	Read: log ship subset every ~10 min for typical clients. Write: RPA, seconds to minutes. At Compassus's 3-hour ship it is too stale for call-offs. No minimum cadence committed.	0:46:46
Common 04 Advise versus decide	• PARTLY	Clear principle: the scheduler decides; every suggestion is accept or dismiss; overrides need a reason. No customer named who moved the line. HCHB task auto-clearing is "planned in the next few weeks."	0:59:04
Common 05 The clinician says no	• PARTLY	Decline in the app will feed the Call-Off Center (future). Override reasons are learned. No 60-day model change was described, and there is no clinician-facing rationale.	1:15:08
Common 06 A Tuesday that goes wrong	• PARTLY	Described through Call-Off Center slides, not run live. "Nobody takes the visit" was not addressed.	1:05:33
Common 07 What you don't do	• PARTLY	Volunteered gaps: authorization and patient communication. No dates given. They were upfront that they were "not selling futures."	1:21:32
Common 08 What breaks at our size	• PARTLY	Largest client 3,500 ADC. Sync fully live at 3 organizations covering ~6-7k patients, plus one in beta. They said scale slows the data pipeline. Nothing said about what broke.	1:50:25
Common 09 The one number you'd stake it on	• NOT ASKED	No measured result was presented.	—
Common 10 Where you'll look worse	• NOT ASKED	They volunteered it anyway: hospice-first, with a home health build ahead.	0:23:57

QUESTION	STATUS	WHAT WE HEARD	WHEN
VitalisCare 01 Home health versus hospice; demo a home health branch	● ANSWERED	Primarily hospice. The demo was hospice data throughout.	0:23:57
VitalisCare 02 Therapy disciplines	● ANSWERED	No therapy model today. Productivity by HCHB points or visit count per discipline; therapy would be customization.	1:33:55
VitalisCare 03 Walk a 60-day Medicare episode	● NOT ANSWERED	Not attempted. "Benefit periods, episodes" was mentioned as a terminology difference only.	1:35:49
VitalisCare 04 Patient communication plan	● ANSWERED	None exists. It was mapped earlier and deprioritized in hospice (HIPAA, relationship risk). "Very within scope." No date or cost.	1:29:00
VitalisCare 05 Keeping not-ready visits off the schedule	● ANSWERED	They can't, beyond flags on auth windows from HCHB data. Readiness is not a state. Intake plots visits in HCHB today.	1:22:52
VitalisCare 06 Point-weighting and branch roll-up dates	● PARTLY	Points come from HCHB or visit count. Roll-up is "next." "Weeks, not months," with no dates or funding model.	1:20:51
VitalisCare 07 Company size and funding	● NOT ASKED	Not covered. They said Sync is their largest investment of time and effort.	1:55:48
VitalisCare 08 Product set and what clinicians install	● PARTLY	Sync (scheduler web), Sync360 (clinician mobile and GPS tracking), Account Portal and SSVI Predictor. Clinicians would run Sync360 alongside PointCare.	1:13:48

What to weigh

HIGH

We would be the home health design partner

Authorization, episode and LUPA, therapy and engagement all need building. The upside is influence and speed. The cost is that our team specifies it and our branches find the bugs.

HIGH

Data cadence is ours to fix

The 3-hour HCHB log ship undermines same-day recovery. Options: a faster ship, a subset script on our side, a database mirror, the data modernization program, or their HCHB partnership in 2027.

HIGH

Exceptions are not shipped

The Call-Off Center is the component that makes everything else work with lagged data. It is due before year end, and slides disagree on its status.

MEDIUM

RPA write-back fragility

Robots working HCHB's screens break when HCHB changes. We didn't ask about detection, repair SLA or production incidents.

MEDIUM

GPS tracking and clinician trust

Route adherence and mileage discrepancy reports are oversight tools. For salaried, tenured clinicians they are a change-management and HR question as much as a feature.

MEDIUM

HCHB still in the loop

Visit plotting stays in HCHB with intake. Scheduler workflow tasks pile up unless auto-cleared (planned). This conflicts with our north star of no HCHB touch points.

MEDIUM

Scale and durability unknown

3 live organizations and ~6-7k patients against our footprint. Company size, funding and headcount were not covered. We would likely be their largest customer by a multiple.

MEDIUM

Security and continuity unpresented

Architecture & Continuity was on their agenda and skipped for time. Uptime, SOC 2 and their HIPAA posture are open.

IN THEIR FAVOUR

Philosophy that fits us

Advise-not-decide, reasons on every override, and incremental change over big reshuffles. That matches our Assist boundary and discovery findings on clinician adoption. The team was candid and quick.

What happens now

Agreed on the call

1. VITALIS

Send the PDF of the Sync-Compassus deck (45 slides; only about a quarter were shown).

2. COMPASSUS

Share the meeting recording with Vitalis (they asked, since Compassus owned the invite).

3. COMPASSUS

Send deep-dive topics ahead of a second virtual call, targeted for the week of Oct 5. Most of the Vitalis team is out until Oct 5.

4. COMPASSUS

Look at raising HCHB log-ship frequency beyond the current 3 hours.

5. COMPASSUS

Decide who comes to the on-site summit in Tennessee, week of Oct 19.

Recommended for the second call

- Walk one Medicare episode end to end on home health data: SOC window, frequency, recert and LUPA flag. This is our shared scenario.
- Written scope, sequence and dates for the home health build: authorization and readiness, episode and LUPA, therapy, engagement, territory map. Who funds it and what the design-partner terms are.
- Their minimum HCHB cadence per feature, and the RPA break-and-repair story with a production incident.
- Run the Tuesday call-off live, including when nobody takes the visit.
- Architecture & Continuity: uptime history and commitment, SOC 2, HIPAA posture.
- Company size, funding, headcount, and our share of their revenue.
- One measured result from a named client, with baseline and period.
- Clinician-facing assignment rationale and day-before confirmation: prototype or date.

Who was there

Compassus

Michael Thompson

VP, Operational Excellence

Colin Highland

Initiative lead · PT, ~25 years in home health

Evan Kramer

Leads Innovation & Operational Excellence

Paige Huffman

Senior Project Manager, Operational Excellence

VitalisCare

Dina Yankelwitz

CEO · led the demo

Chavi Bornstein

Head of Product

Faigy Stroh

Director of Customer Success

Malka Hess

Data engineering · HCHB pipeline

Rachel Greenhut

Security, infrastructure, HIPAA and SOC

Bob Willix

VP Commercial Growth · RN

Shira Rubinstein

Product Owner, Sync · Call-Off Center

R. Feldman

Lead Architect, algorithms

Shayna Fishman

Sync360 · tracking and verification

Esther

Development and technology operations

Built from the Teams recording (2:00:32) with a machine transcript and extracted screens. Speaker attribution is inferred from context where the transcript has no labels. Timestamps are h:mm:ss into the recording.

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